Fax cover sheet

Canyon Oaks Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Brian Castillo, RN CM
Phone: 555-0190
Date: 08/14/2026 11:30
Pages (including cover): 13
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

79F s/p L hip IMN 08/11, WBAT. Pt independent with ADLs.
Pt prefers recuperative care over SNF. Please review.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Canyon Oaks Medical Center (fictional)
Department/Floor: 4 Ortho
Social Worker/Case Manager/Nurse: Brian Castillo, RN CM
Phone: 555-0190
Email: bcastillo@canyonoaks.example
Nursing Station: 555-0191
Authorized By: Dr. T. Iwata (Ortho)

Patient Information

Name: Eleanor Doe
MRN: CO-3398420
Birth Date: 03/09/1947
Gender: F
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal/Medicare
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: L hip fracture s/p ORIF
Admit Date: 08/10/2026
Any Wounds: Yes [X] No [ ]
Number/Location/Size/Stage: 1 - L lateral hip incisions x2 with staples
Independent with Wound Care: Yes [X] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 08/10/2026 Result: CXR neg

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [X] Yes [ ]

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CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Eleanor Doe
DOB: 03/09/1947
Member ID: OCH-11908326

Referring Provider

Facility: Canyon Oaks Medical Center (fictional)

Clinical

Primary Diagnosis: Intertrochanteric fracture of left femur, s/p fixation
ICD-10: S72.142D
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 08/15/2026
Requested Days: 30

Plan Use Only

Status: Pending review

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Patient face sheet

Canyon Oaks Medical Center (fictional) - PATIENT FACE SHEET

Patient: Eleanor Doe
MRN: CO-3398420
Account #: 99398420
DOB: 03/09/1947
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 08/10/2026 15:40
Admit Type: Inpatient
Unit/Bed: 4OR-412
Attending: Takeshi Iwata, MD (Orthopedics)

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 08/10/2026

CC

L hip pain after fall

HPI

79 y/o F, unhoused (lives in her car, car towed 07/2026, now at bus shelter), tripped on curb at bus stop and fell onto L side. Unable to bear weight. No LOC, no head strike.
XR: L intertrochanteric femur fracture, closed.
Prior level of function: independent ambulation with single-point cane, independent ADLs.

PMH

- HTN
- Osteoporosis
- Hypothyroidism
- Chronic kidney disease stage 3a

Meds PTA

- Lisinopril 10 mg daily
- Levothyroxine 75 mcg daily

Allergies

NKDA

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History and Physical - page 2

Exam

T 98.2F HR 92 BP 158/84 RR 18 SpO2 97% RA
L LE shortened and externally rotated, NV intact distally
AOx4

Assessment/Plan

1. Closed L intertrochanteric fracture - ortho to OR 08/11 for IMN; pain control; NPO after MN
2. HTN - continue lisinopril
3. CKD 3a - renally dose meds
4. Osteoporosis - post-op calcium/vit D, bone health f/u
5. Dispo - PT/OT post-op; likely SNF

Signed: Takeshi Iwata, MD 08/10/2026 18:20

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Operative note (brief)

BRIEF OPERATIVE NOTE

Date: 08/11/2026
Procedure: Cephalomedullary nail fixation, L intertrochanteric femur fracture (closed)
Incisions: L lateral hip (2), staples
EBL 150 mL. Complications: none
Post-op: WBAT LLE, posterior hip precautions not required
VTE ppx: enoxaparin 40 mg SC daily x 28 days
Staple removal: Ortho clinic 08/25/2026
Surgeon: Takeshi Iwata, MD

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Ortho progress note POD3

ORTHOPEDIC PROGRESS NOTE - POD#3

Date: 08/14/2026 07:00
S: Pain 5/10 with movement, 2/10 at rest. Nauseated with oxycodone yesterday.
O: T 98.6F HR 86 BP 134/76. Incision C/D/I, staples intact. NV intact.
Hgb 9.1 (from 12.4 pre-op), stable x 24h.
A/P: POD3 s/p IMN L hip.
- WBAT, PT/OT BID
- PT recommends SNF; CM working on SNF referrals. Pt reluctant, requesting recuperative care.
- Medically stable for d/c to appropriate level of care.
Signed: A. Kowalski, PA-C

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Physical therapy re-evaluation

PHYSICAL THERAPY - POD3 RE-EVALUATION

Date: 08/13/2026 14:15
Bed mobility: supine->sit Mod A x1, assist for LLE management
Sit<>stand from bed: Mod A x1 with FWW, verbal cues for hand placement
Bed->chair stand-pivot: Mod A x2 (08/12); Min-Mod A x1 (08/13)
Gait: 15 ft with FWW, Mod A x1, w/c follow; limited by pain and fatigue, knee buckling x1
Toilet transfer: Mod A x1-2
Balance: poor dynamic standing, high fall risk

Assessment

Pt is significantly below baseline (PLOF independent with cane). Requires 1-2 person assist for transfers and toileting.

Recommendation

SNF for continued rehab. Not safe for d/c to unsupervised setting at this time.
Signed: M. Delgado, PT, DPT

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Occupational therapy evaluation

OCCUPATIONAL THERAPY EVALUATION

Date: 08/13/2026 10:00
Grooming: Set-up, seated
UB dressing: Min A
LB dressing: Max A (unable to reach feet, no AE training yet)
Toileting hygiene: Max A
Toilet transfer: Mod A x1 with grab bar
Shower: not attempted; anticipate Mod A with tub bench
Cognition: AOx4, good safety awareness

Recommendation

SNF. Will need AE training (reacher, sock aid) and toileting retraining.
Signed: K. Huang, OTR/L

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Nursing note

NURSING SHIFT NOTE

Date: 08/14/2026 06:45
Night shift: pt up to bedside commode x2 with 2-person assist and FWW. Unable to perform own peri-care; RN assisted.
Episode of urinary incontinence 03:00 before assistance arrived - linen change. External female catheter (PureWick) placed overnight per pt request.
Pain 6/10 with transfers, oxycodone 2.5 mg given 03:15.
Bed alarm on, fall precautions.
Signed: R. Oyelaran, RN

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Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
08/13 08:00 | 98.7 | 90 | 138/78 | 18 | 96% | RA
08/13 16:00 | 99.0 | 88 | 132/74 | 18 | 96% | RA
08/13 23:00 | 98.4 | 84 | 128/72 | 16 | 97% | RA
08/14 04:00 | 98.3 | 86 | 130/74 | 16 | 96% | RA
08/14 07:00 | 98.6 | 86 | 134/76 | 16 | 97% | RA

LABS

08/10 | Hgb 12.4 | Plt 228 | Cr 1.2 | eGFR 46 | TSH 3.4
08/13 | Hgb 9.0 L | Cr 1.1
08/14 | Hgb 9.1 L | Cr 1.1 | Na 137 | K 4.3
08/10 MRSA nares PCR: Negative
08/14 05:00 SARS-CoV-2 rapid antigen: Negative

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Discharge medication list

DISCHARGE MEDICATION LIST (DRAFT)

Patient: Eleanor Doe MRN: CO-3398420

- Enoxaparin 40 mg SC daily through 09/08/2026 (pt has not yet self-injected)
- Acetaminophen 1000 mg PO every 8 hours (max 3 g/day)
- Oxycodone 2.5 mg PO every 6 hours PRN severe pain
- Docusate 100 mg PO BID
- Senna 8.6 mg 2 tabs PO HS PRN
- Calcium carbonate 600 mg + vitamin D3 800 IU PO BID
- Lisinopril 10 mg PO daily
- Levothyroxine 75 mcg PO daily

DME: FWW, bedside commode, tub transfer bench (per OT)

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